

Screening logic to care pathways (no diagnosis inference)



Uncertain symptom patterns and context



Risk screening $u(x)$
(red flags / unstable vitals)



TiTrATE-based routing
(timing / triggers / targeted examination)



Care pathway outputs (G1–G5)

Emergency / observation / low-acuity / self-care

Rationale and traceability
(rule paths, uncertainty flags, logs)

Governance and service management
(metrics, audits, policy control)